Vancouver Coastal Health

Turning every contract into a promise of better care



Vision Consulting

Executive Summary | VCH should implement a scalable, risk-tiered framework for proactive contract oversight to achieve 14M cumulative cost savings

Vancouver CoastalHealth

Question

How can VCH design and implement a scalable, culturally safe contract and risk management framework for its diverse network of partners?

Solution

A risk-tiered framework that enables scalable, proactive contract oversight

Visibility	**Consistency**	**Handling**
Impact oriented portfolio prioritization system	Standardizing KPI Categories across portfolios	Creating a risk alert and an active governance system

KPIs

13K+	**$14M**	**250+**
Admin hours saved	Cumulative Cost Savings	Risk Incidents Predicted

Overview | VCH needs a realistic and scalable contract risk framework that takes the organizations key constraints into consideration

Current Situation

- VCH is facing a **growing volume and complexity of contracts**[1]
- VCH's contract management process is currently **fragmented, reactive, and manual**[1]

Goal

Design and implement a **scalable, culturally safe contract** and **risk management framework** that allows integration of diverse service providers

Core Competencies



Experienced CMO



Strong Cross Functional Teams



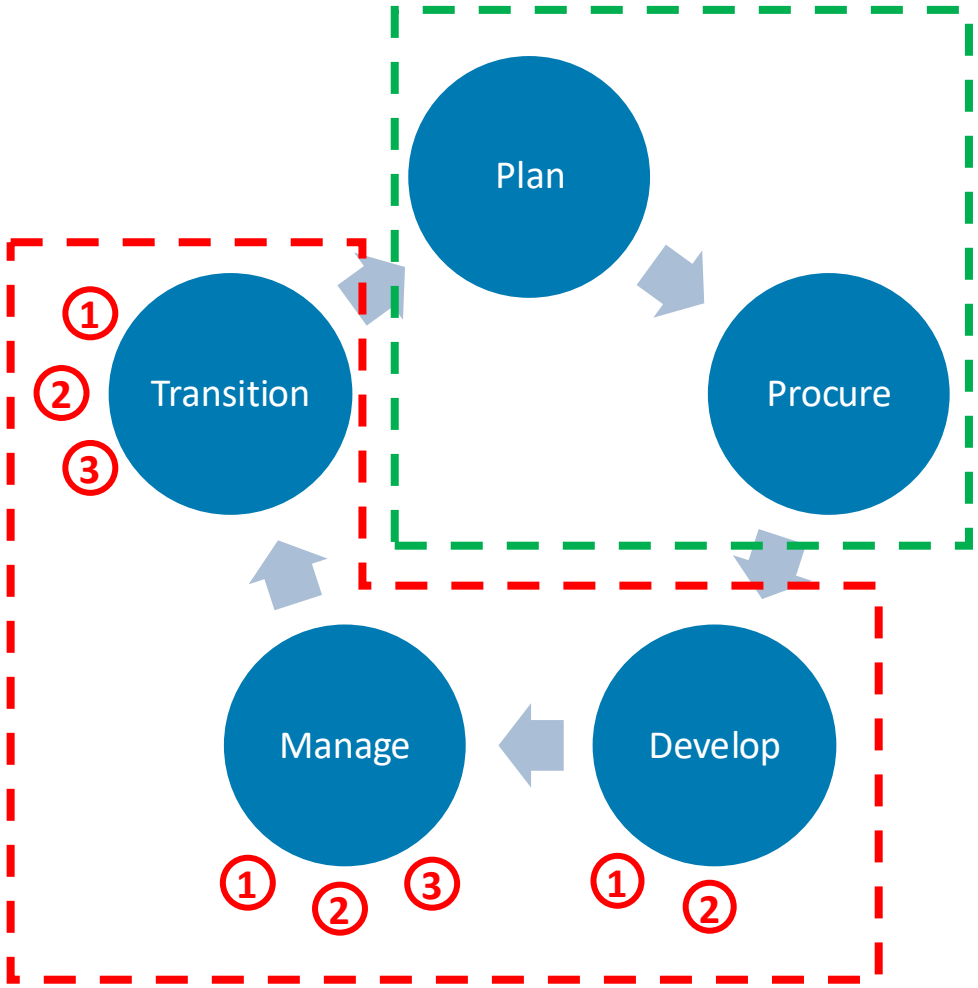
Oracle CLMS

Key Considerations

1. **Financial Constraints**
2. **Technology Readiness**
3. **Cultural Safety**
4. **Strategic Alignment**

Sources: [1]Case

Overview | VCH's current challenges lie in unclear prioritization, lack of portfolio level analysis and assessment, and a reactive risk management style



1 Unclear Prioritization

VCH lacks a scalable approach to contract prioritization as volume and complexity grow[1]

2 Lack of Portfolio Level Visibility

VCH lacks consistent reporting and comparability across contracts, limiting portfolio-level insights[1]

3 Reactive Risk Management

VCH's current risk management approach is largely reactive, increasing the likelihood of incidents[2]

VCH's main challenges lie within the develop, manage, and transition steps

Sources: [1]Case, [2]EHS Insight

Overview | VCH's framework should address their current challenges while keeping their 4 key considerations in mind

	Strategies Considered	Financial Constraints	Cultural Safety	Technology Readiness	Strategic Alignment
Scalability and Prioritization Ability	Impact oriented portfolio prioritization system	●	●	●	●
	Industry segmented portfolio system	●	●	●	●
	Region segmented portfolio system	●	●	●	●
Portfolio Level Visibility	Standardized KPI categories across portfolios	●	●	●	●
	Predictive AI insights across contracts	●	●	●	●
Proactive Risk Management	Risk alerts and active governance	●	●	●	●
	Predictive risk management with AI	●	●	●	●
	Manual weekly CM supplier check-ins	●	●	●	●

Overview | An impact-oriented framework that enables scalable, proactive contract oversight best addresses the challenges VCH currently faces

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Challenges

1 **Unclear Prioritization**
VCH lacks a scalable approach to contract prioritization as volume and complexity grow

2 **Lack of Portfolio Level Visibility**
VCH lacks consistent reporting and comparability across contracts, limiting portfolio-level insights

3 **Reactive Risk Management**
VCH's current risk management approach is largely reactive, increasing the likelihood of incidents

Framework Elements

1 **Visibility**
Classifying contracts into portfolios based on impact-oriented metrics

2 **Consistency**
Creating standardized KPI categories to compare contracts and create visibility at a portfolio level

3 **Handling**
Creating a risk system that proactively alerts contracts at risk and defining rules for escalation and governance

Statistics

Portfolio-managed organizations complete **35% more programs successfully**[1]

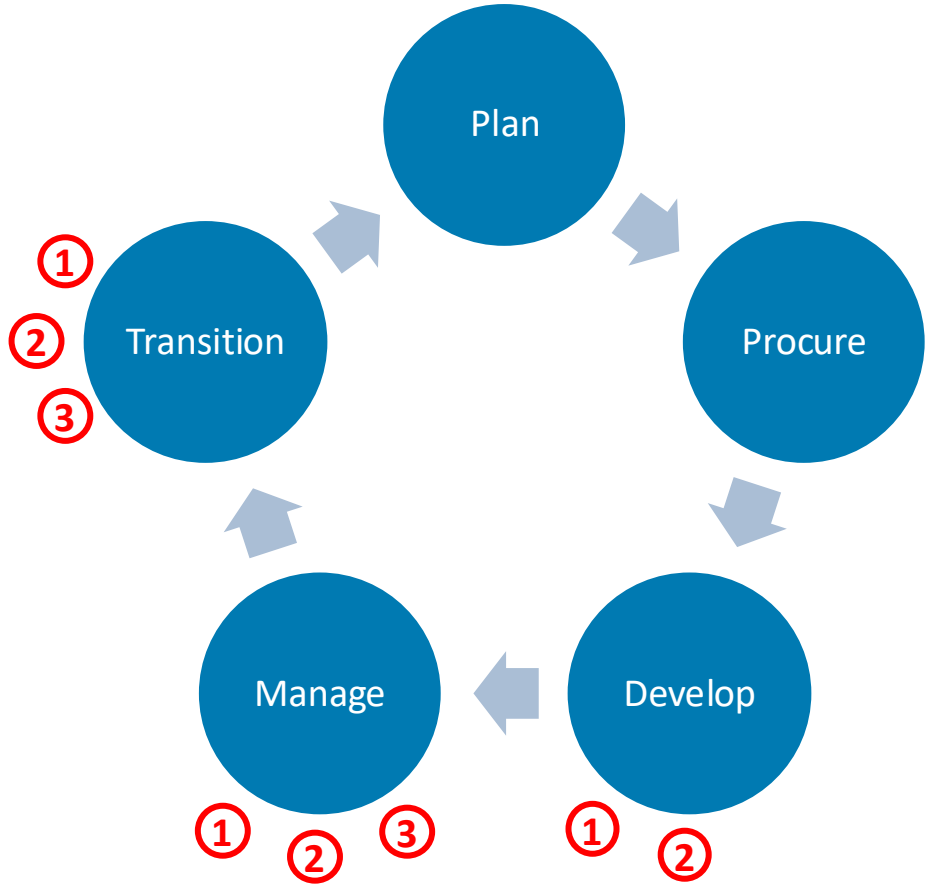
Standardized metrics significantly **reduce risk of conflicting data interpretations**[2]

Proactive safety interventions can **reduce adverse events by up to 50% in healthcare**[3]

Sources: [1]Project Management Institute, [2]IJIRMPS, [3]BMJ Journals

Service Provider	Vancouver Aboriginal Health Society	Veolia Canada
Service Provided	Provides culturally appropriate primary care, and other services for Indigenous populations	Medical waste management, environmental services, and facility support operations
Contract Size	$2 Million	$30 Million
Geographical Region	Urban Vancouver	Regional across VCH facilities
Intended Serviced Population Addressed	Indigenous Individuals, particularly those that are underserved	Supports entire patient population through operational infrastructure

Visibility | Addressing unclear contract prioritization process with an impact-oriented portfolio prioritization system



1 Unclear Prioritization

VCH lacks a scalable approach to contract prioritization as volume and complexity grow[1]

2 Lack of Portfolio Level Visibility

VCH lacks consistent reporting and comparability across contracts, limiting portfolio-level insights[1]

3 Reactive Risk Management

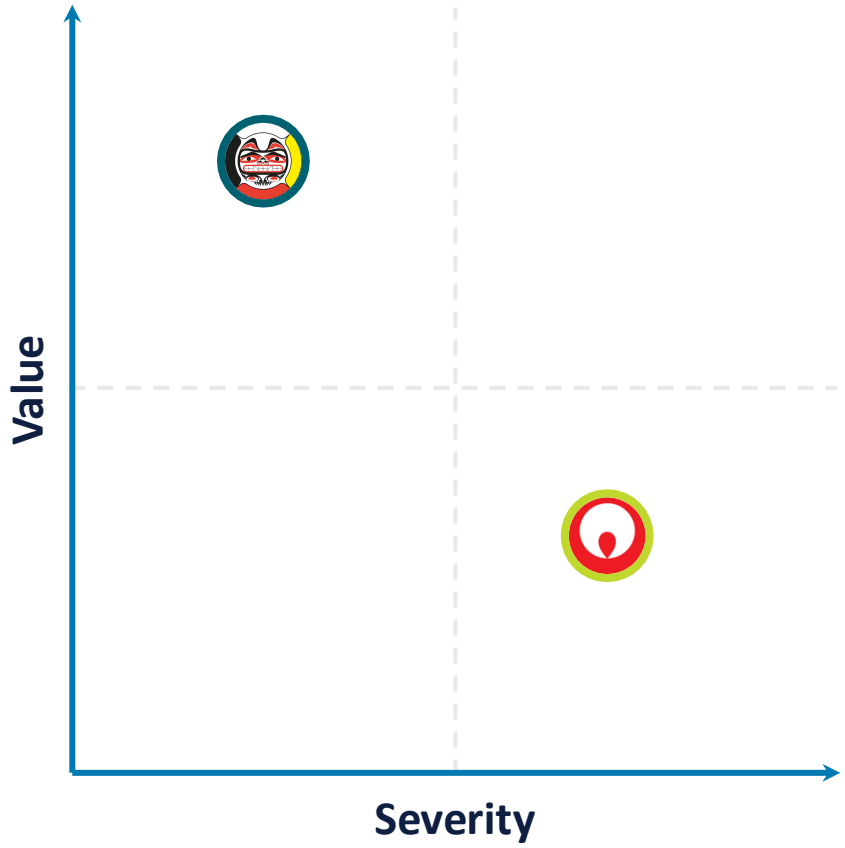
VCH's current risk management approach is largely reactive, increasing the likelihood of incidents[2]

VCH's main challenges lie within the develop, manage, and transition steps

Sources: [1]Case, [2]EHS Insight

Visibility | VCH should establish a standardized scoring framework based on severity and value to prioritize contract oversight and risk

Strategic Objective Prioritize oversight by contract severity and value for better time allocation



A weighted score assigned from 1 to 5, standardized organization-wide[2]

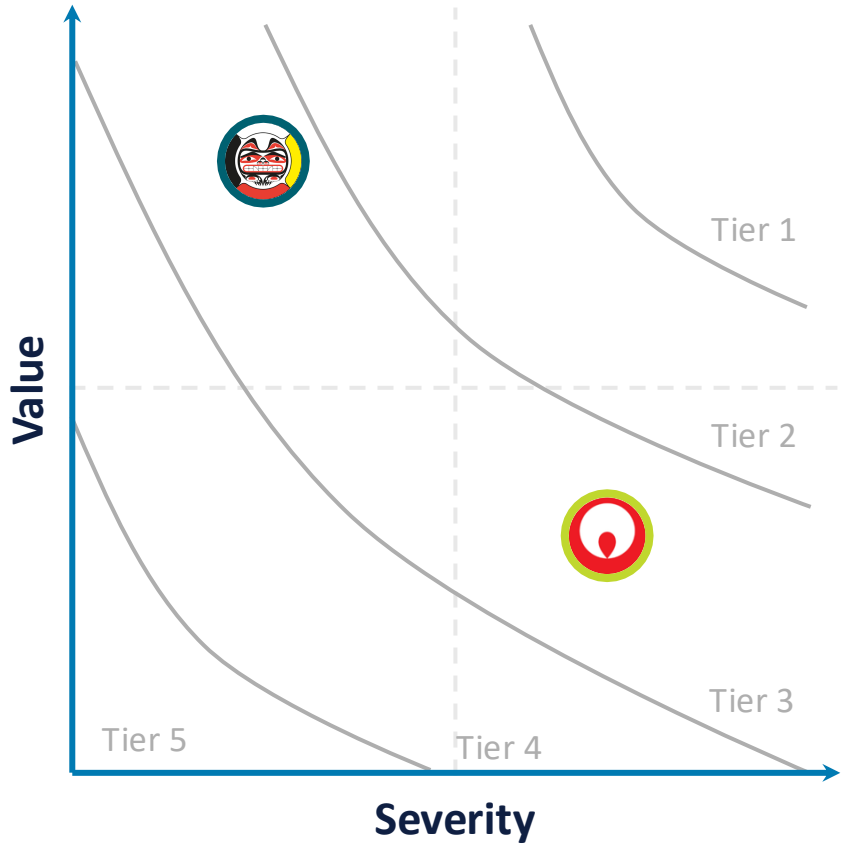
Severity Factors	VAHS	Veolia
Non-compliance risk (25%)	Weighted score: 1.75	Weighted score: 3.20
Performance risk (20%)		
Procurement and legal risk (20%)		
Financial risk (20%)		
Reputational risk (15%)		

Value Factors	VAHS	Veolia
Strategic importance (35%)	Weighted score: 3.00	Weighted score: 1.70
Population impact (35%)		
Provider criticality (30%)		

Sources: [1]PwC, [2]Appendix 3

Visibility | Leverage the risk-tiered contract prioritization model to prioritize contract oversight and risk

Strategic Objective Prioritize oversight by contract severity and value for better time allocation



Indication of Tiered Portfolios

Time allocated to monitoring contracts

Prioritization of tasks/ contracts

Decision-Making Team

Contract advisors

Finance team

Operations team

Legal team

Subject matter experts (SMEs)

Visibility | VCH should delegate SMEs to proactively ensure Indigenous and local service providers are not disadvantaged in scoring

Who are SMEs?

Experienced Advisors[1] — Strong CMOs or operational leaders within VCH



\+

Passionate CMOs[1] — Passionate about aiding indigenous and local community



SME Team — Team of 3 CMO SMEs dedicated to aiding indigenous cases

SME Responsibilities[2]

1. **Validate tiers** for Indigenous or local service providers before they are finalized in CLMS
2. **Champion culturally safe, equitable practices** via appropriate language, etc.
3. **Support complex contract decisions and escalations** by joining necessary discussions

SME Training

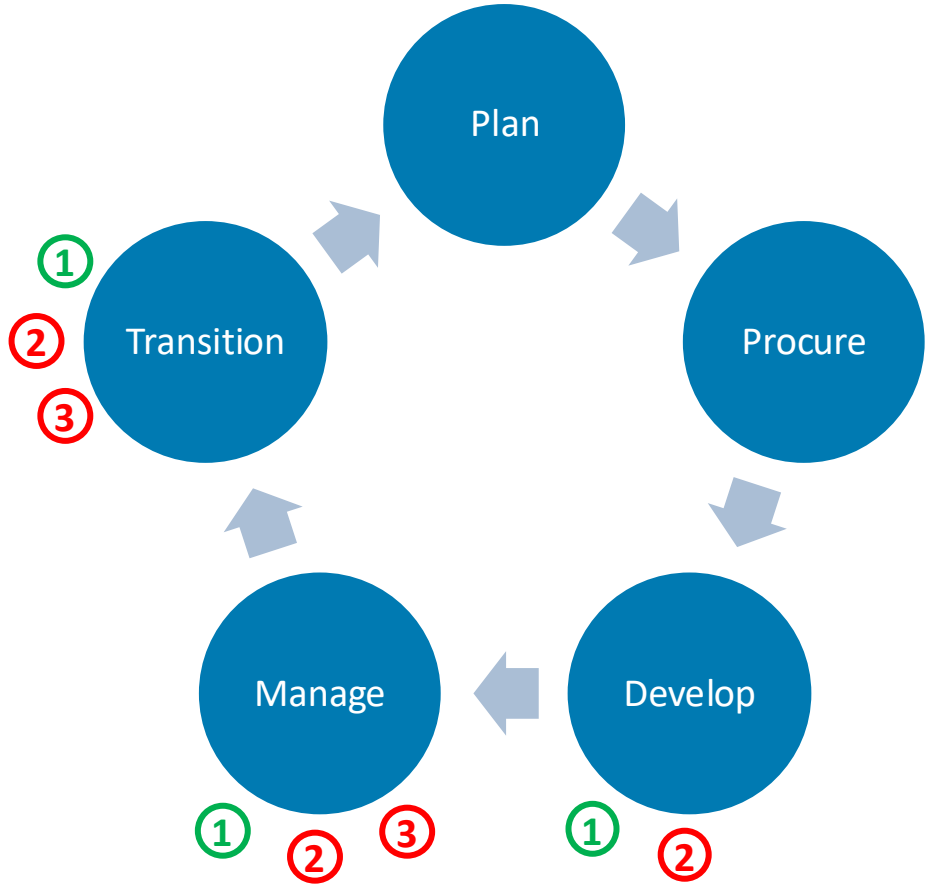
SMEs will be selected via nomination from leaders and volunteering

- Cultural safety virtual workshop[3]
- Governance and role playbook[3]

Keep admin burden flat while improving cultural safety by **formalizing existing Indigenous advisory work.**

Sources: [1]Coursera, [2]University of Waterloo, [3]LinkedIn

Consistency | Classifying contracts into impact-based portfolios create visibility, the next step is to address portfolio level visibility



1 Visibility

Classifying contracts into portfolios based on impact-oriented metrics

2 Lack of Portfolio Level Visibility

VCH lacks consistent reporting and comparability across contracts, limiting portfolio-level insights[1]

3 Reactive Risk Management

VCH's current risk management approach is largely reactive, increasing the likelihood of incidents[2]

VCH's main challenges lie within the develop, manage, and transition steps

Sources: [1]Case, [2]EHS Insight

Consistency | To ensure independent contracts are comparable, VCH should require KPIs within standard categories to measure them in a consistent way across providers

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Strategic Objective Clear, consistent success metrics across all currently independently managed contracts

KPI Categories:	VAHS	Veolia Canada
Service Delivery Timely, accessible service for the intended population (appointments, hours, reach)[1]	% of patients who can get an appointment within 7 days[4]	Average response time to maintenance requests[6]
Quality Clinical/service outcomes and experience, including cultural safety and satisfaction[1]	A small Indigenous clinic like VAHS prioritizes[5]: 1. Culturally safe access 2. Holistic wellness KPIs	A large facilities vendor like Veolia emphasizes at scale[6]: 1. Reliability 2. Cost efficiency 3. Safety and environmental compliance
Financial Efficient, transparent use of funds (cost, budget adherence, unit cost)[2]		
Compliance Adherence to contract terms, reporting, privacy, and legal/clinical standards[3]		

Sources: [1]Sage, [2]Aeon, [3]Netsuite, [4]Vancouver Aboriginal Health Society, [5]Indigenous Cultural Safety Trainings for Healthcare Professionals Working, [6]LinkedIn, [7]Veolio

Consistency | Automate KPI extraction from contracts using OCI Document Understanding to enable consistent and scalable performance tracking

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Strategic Objective Reduce manual data entry and enable consistent, portfolio-wide performance tracking

Document Understanding

ORACLE Cloud Infrastructure

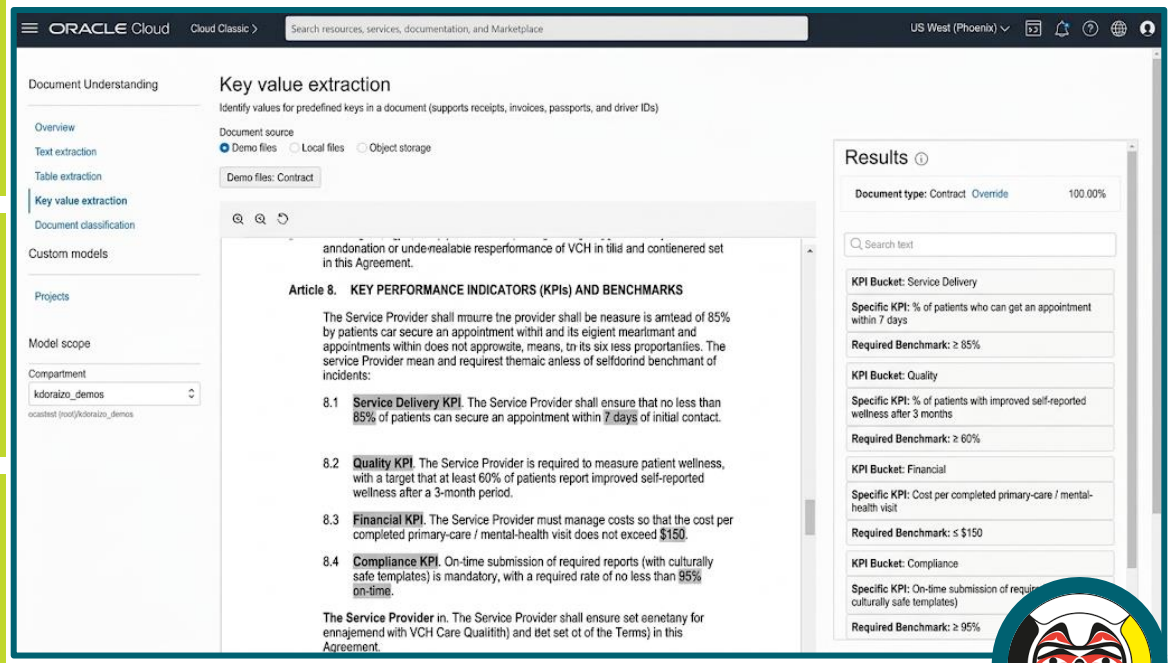
Oracle Cloud Infrastructure (OCI) Document Understanding reads contract documents and auto-populates KPI fields in CLMS[1]

Text Extraction Process

Standardize contract templates so KPIs appear in the same section

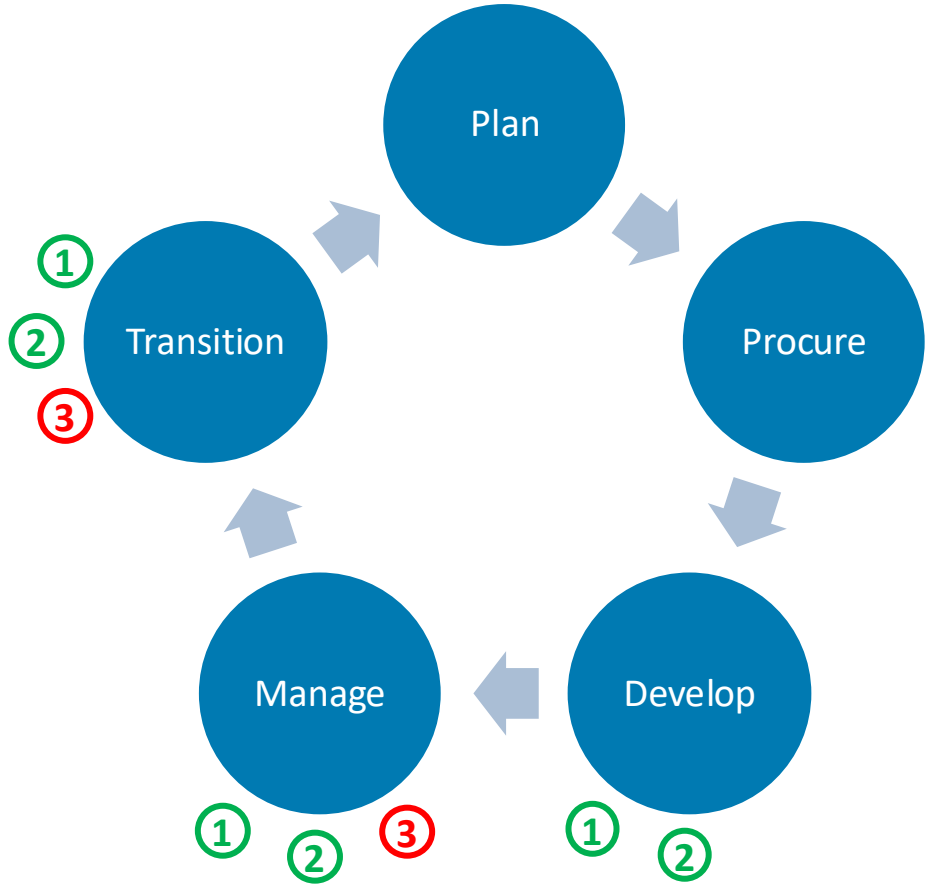
Service provider uploads final contract on CLMS

Software extracts key-value pairs and maps them in CLMS



Sources: [1]Oracle, [2]Image generated with the aid of Nano Banana

Handling | Creating standardized KPI categories allows VCH to compare contracts and create visibility at scale, the next step is to create a proactive risk approach



1 Visibility

Classifying contracts into portfolios based on impact-oriented metrics

2 Consistency

Creating standardized KPI categories to compare contracts and create visibility at a portfolio level

3 Reactive Risk Management

VCH's current risk management approach is largely reactive, increasing the likelihood of incidents[2]

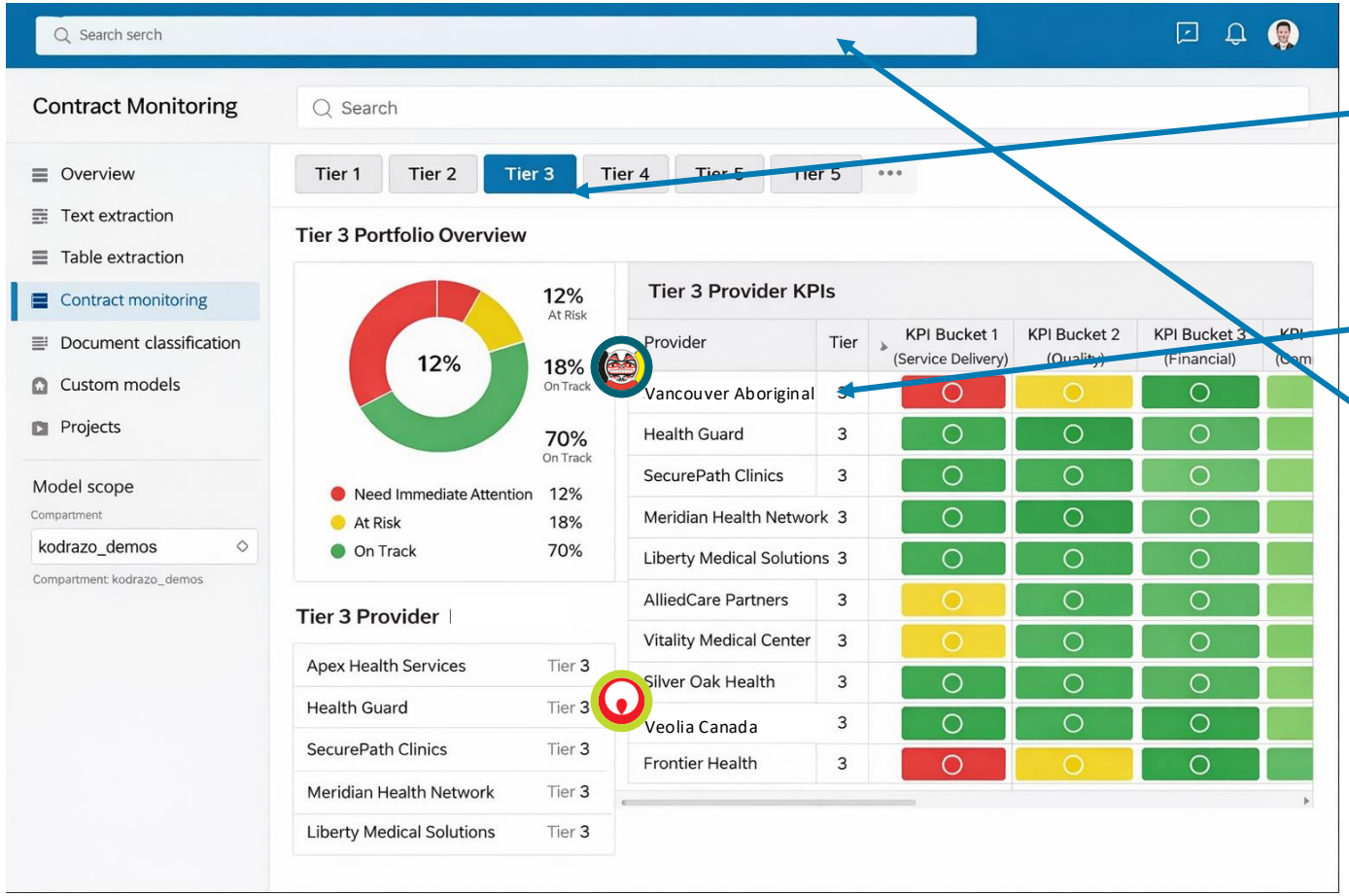
VCH's main challenges lie within the develop, manage, and transition steps

Sources: [1]Case, [2]EHS Insight

Handling | Using risk dashboards and alerts to enable proactive prioritization of at-risk contracts before crises occur

Strategic Objective

Proactive approach to visualize portfolio-level analytics



Portfolio-level overview of contracts to evaluate systemic risk

Proactive alerts based on KPIs falling out of the acceptable threshold

Look at certain contracts or service providers in depth

Shifts VCH from reactive, contract-level monitoring to **proactive, portfolio-level risk management**

Sources: [1]Oracle, [2]Image generated with the aid of Nano Banana

Handling | VCH should outline a 2-step escalation and governance pathway

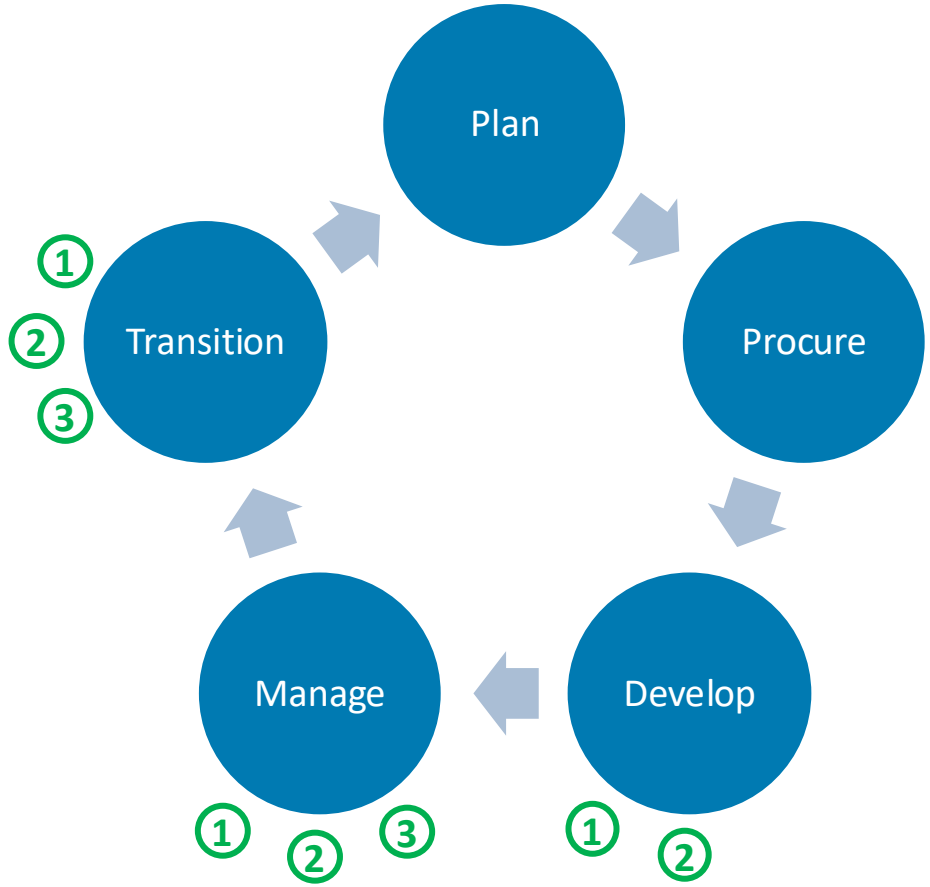
Alert Trigger	1. CM (Operations)	2. CA (CMO)
		
Alert is triggered in the CLMS dashboard	CMs are notified as first responders and have one recording cycle to take action	If alert continues after another recording cycle, CA is notified
Expected Response[1]	1. Review details and contract context 2. Contact provider to learn cause 3. Align on corrective action plan 4. Monitor progress till next reporting	1. Review CM's actions and risk left 2. Decide leadership escalation level 3. Agree on response and assign owner/timeline

Specialized teams (Indigenous and Local Service Provider SMEs, Finance, Legal, etc.) support CM at Step 1 and join CA governance at Step 2

Formal role definitions and clear escalation path allows easy, and scalable governance

Sources: [1]Workday

Handling | The V.C.H. strategy addresses current contract management pain points in Develop, Manage, and Transition



1 Visibility

Classifying contracts into portfolios based on impact-oriented metrics

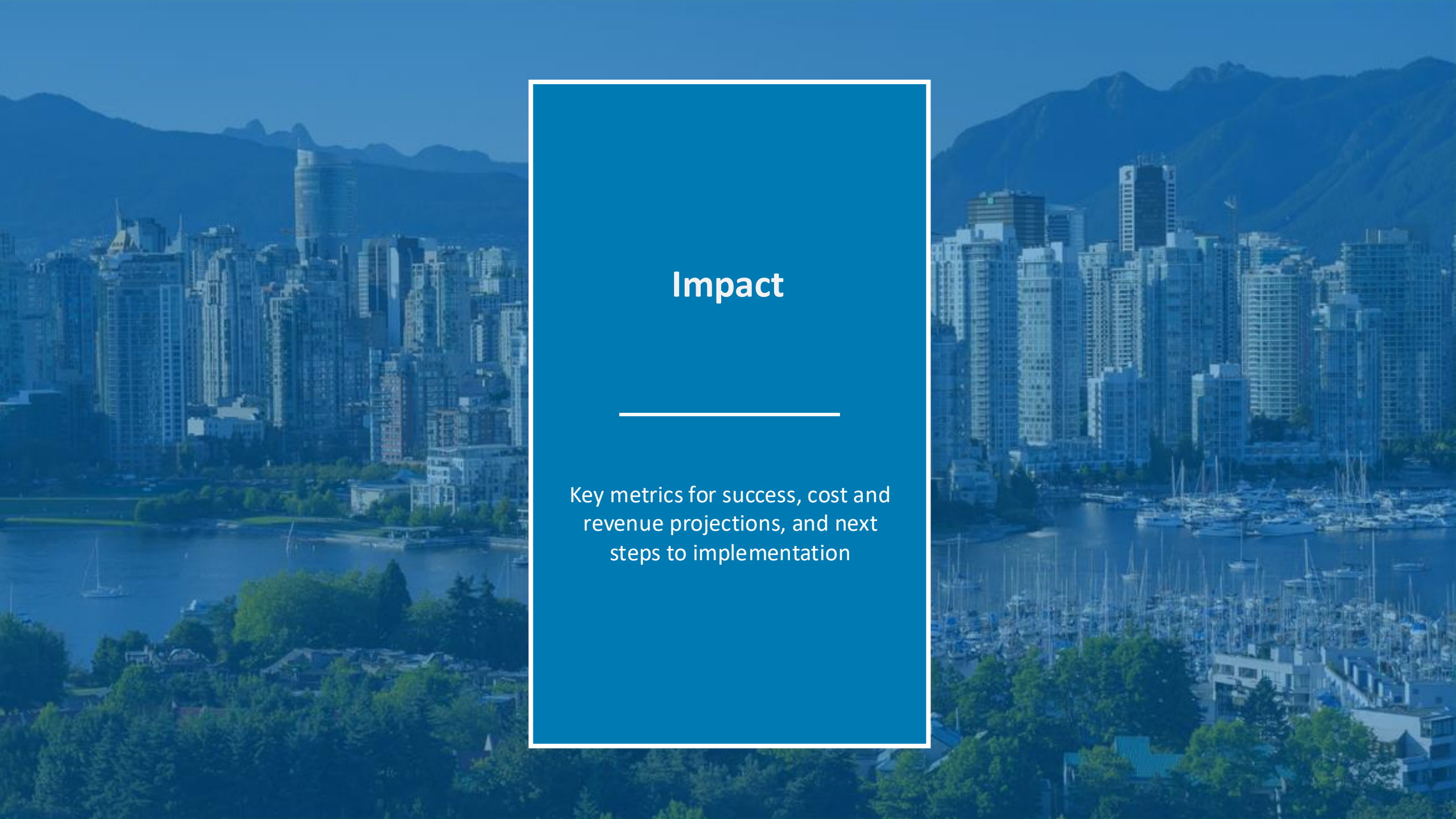
2 Consistency

Creating standardized KPI categories to compare contracts and create visibility at a portfolio level

3 Handling

Creating a risk system automatically alerting contracts at risk and defining rules for escalation and governance

A risk-tiered framework that enables scalable, proactive contract oversight



Impact

Key metrics for success, cost and revenue projections, and next steps to implementation

Impact | To track the success of the V.C.H. strategy, numerous KPIs should be tracked

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Quantitative

Cost Savings



$14M

5Y Cumulative with V.C.H. strategy

ROI



10.27x

ROI from cost savings

Qualitative

Ensured Care



2100+

Indigenous members with trusted care provided by VCH

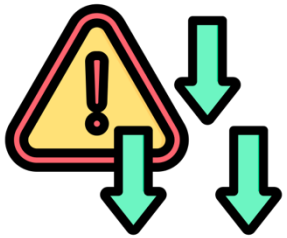
Hours Saved



13K+

Annual Reduction in time spent on metrics tracking for contract managers

Risk Reduction

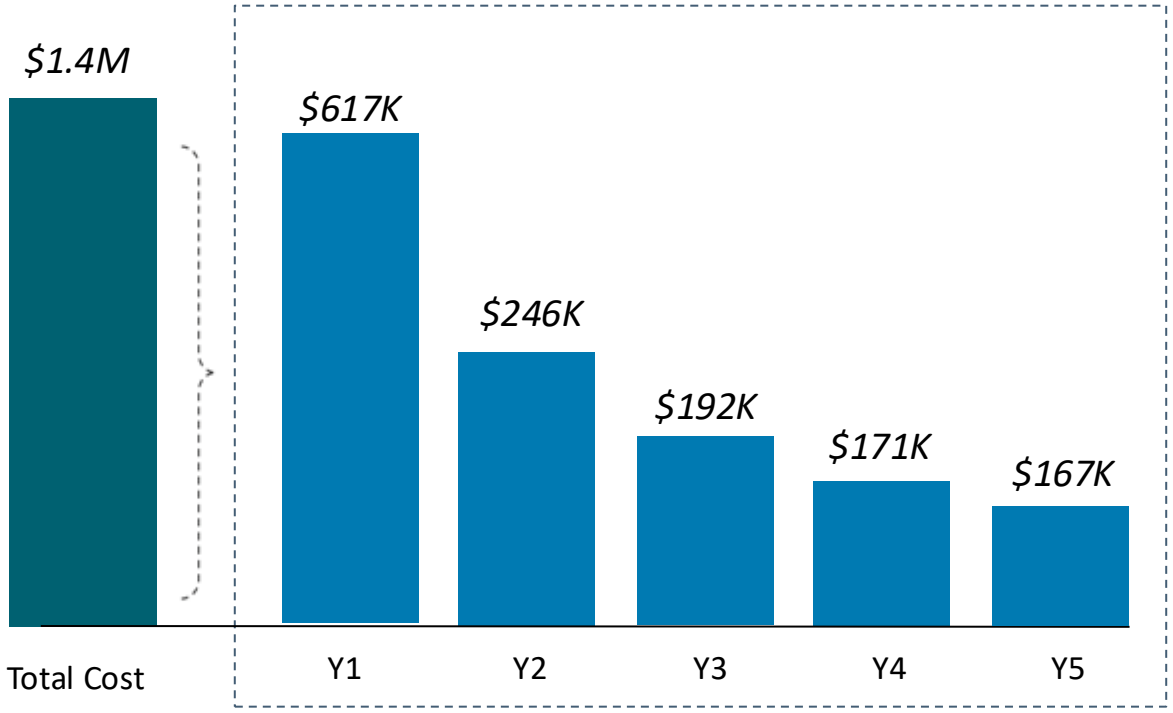


250+

Incidents predicted using V.C.H. framework

Impact | Implementing the V.C.H. strategy requires a total $1.4M investment

Projects Costs ($ CAD)



$342.5K One-Time Build for Dashboard

~$55K In Training of New Dashboard in Y1

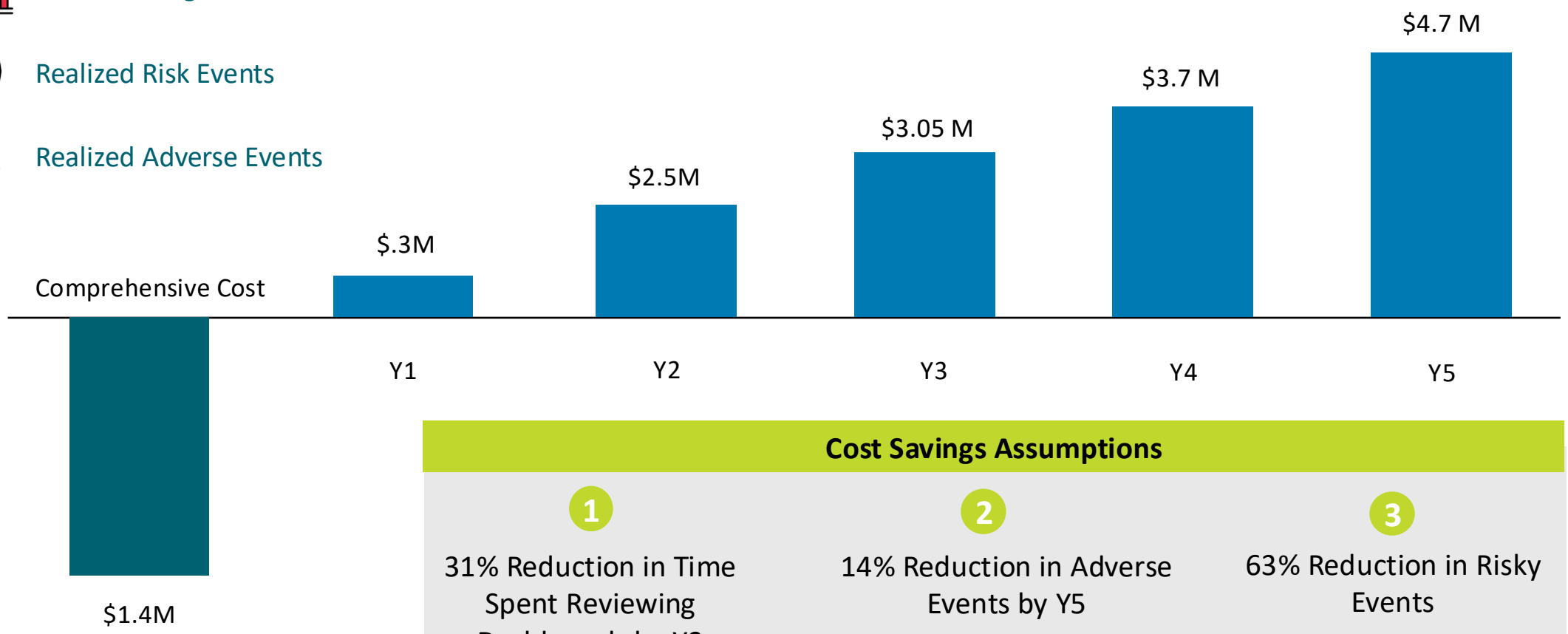
Cost Drivers

- Dashboard Build
- Integration with Existing Systems and Staff + IT
- OCI Setup and Management Fees
- Contract Onboarding
- Training
- Contract Manager FTE
- Legal Compliance & Set Up

Impact | With this strategy, VCH will be able to save $14M cumulatively

Key Drivers

- Admin Savings
- Realized Risk Events
- Realized Adverse Events



Cost Savings Assumptions

1. 31% Reduction in Time Spent Reviewing Dashboards by Y3
2. 14% Reduction in Adverse Events by Y5
3. 63% Reduction in Risky Events

Impact | Over the next 2 years, VCH will implement and pilot the V.C.H. strategy

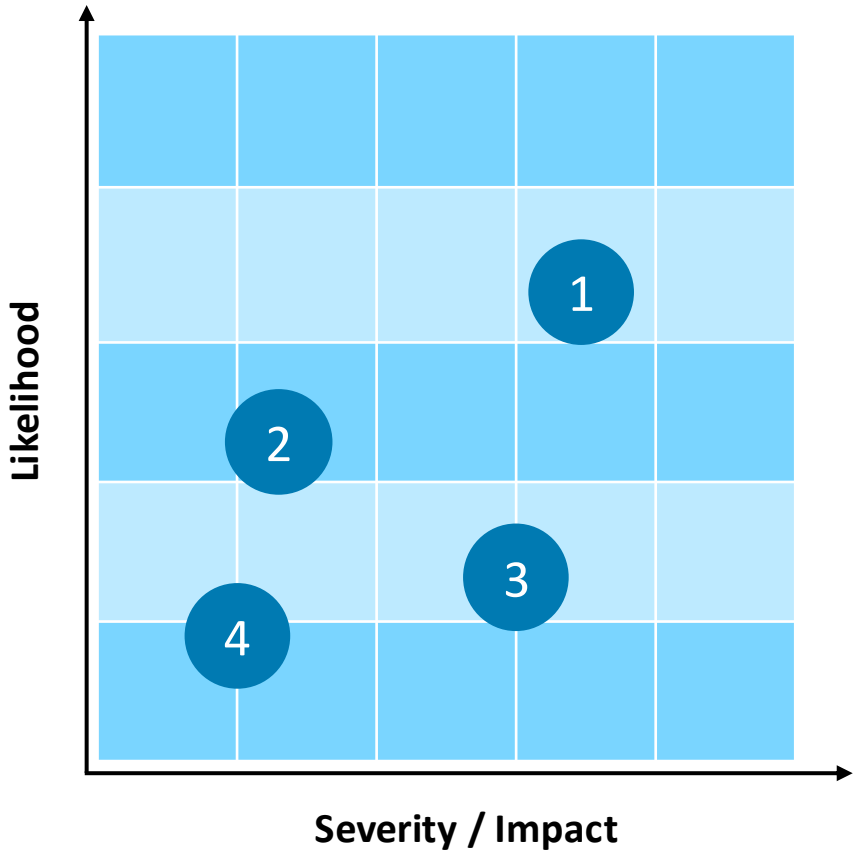
5-Year Timeline

The V.C.H. Strategy	Y1	Y2	Y3	Y4	Y5
Select and train SMEs	■				
Upgrade CLMS software	■	■			
Pilot with 50 contracts		■			
Gather pilot feedback and reiterate			■	■	
Create CMO training and user guides				■	
Rollout to all stakeholders					■
Monitoring and refine system					■

Immediate Next Steps

1. Assemble Indigenous and local service provider SMEs
2. Validate tiering scales and KPI categories
3. Communicate with IT team to configure CLMS

Impact | Four key risks identified with the V.C.H. strategy can be addressed with mitigation strategies



	Risks	Mitigations
1	Alert Fatigue	Review thresholds to ensure they properly demonstrate criticality
2	Risk scoring oversimplifies complex situations	Involve SMEs and human review for cultural and EDI sensitivity involving edge cases
3	Over standardization reduces flexibility	Ensure KPI categories can be universally applied to all KPIs
4	Increased Administrative Burden In Rollout	Phased rollout starting with high risk contracts

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A risk-tiered framework that enables scalable, proactive contract oversight

Visibility	**Consistency**	**Handling**
Impact oriented portfolio prioritization system	Standardizing KPI Categories across portfolios	Creating a risk alert and an active governance system

KPIs

13K+	**14M**	**250+**
Admin hours saved	Cumulative Cost Savings	Risk Incidents Predicted

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Appendix | Improvements to Current De-risking Strategy

Current De-risking Model	Gaps	Our Solution
Compliance Risk List[1] Monitoring & De-risking	No standardized method for tiering; only identifies high-risk contracts[2]	Other tiers are established with metrics clearly defined so they can be standardized across the organization
Financial Monitoring[1] Proactive	Purely financial risk assessment of contracts; no comparable across contracts[3]	Severity and value factors accounted for in segmenting contracts, KPI categorization to give a more holistic view
Financial Crisis Playbook[1] Reactive	Reactive plan to crisis; doesn't follow through with portfolio-wide learning[4]	Allows for contingency planning based on tier of contract, consistent monitoring of KPIs can allow for early detection

Sources: [1]Case, [2]Censinet, [3]PwC, [4]Deloitte

Appendix | Case Studies to Prove Success

	U.S Federal Healthcare Contract Management[1]	Highmark Health[2]	Ohio State University[3]
Problem	The organization faced **fragmented**, paper-based contract management across multiple locations, leading to **lost documents, poor visibility into procurement status, and significant financial risk**	**Fragmented risk management** with siloed teams, duplicative processes, and excessive committees **slowed decision-making and reduced governance effectiveness**	**Lack of standardized contracts** and no central repository led to **poor visibility, inefficient sourcing**, and difficulty tracking expirations
Solution	**Solution:** Implemented a centralized digital repository and workflow tracking system that standardized document management and provided end to end visibility	**Solution:** Implemented a centralized RiskOps model to **standardize risk processes, align governance, and streamline decision-making**	**Solution:** Implemented centralized contract management and **standardized sourcing processes** using Ariba
Result	**Result:** The organization **improved contract accessibility** and tracking, **reduced financial losses**, and **increased operational efficiency** across all locations	**Result:** Reduced cycle times, eliminated duplication, and **improved risk visibility and decision quality.**	**Result:** Achieved ~$1M in savings, **improved visibility and collaboration**, and enabled **proactive contract tracking and governance**

Sources: [1]DocuVantage, [2]ISACA, [3]Slideshare

Standardized Scoring Framework

1 Severity and Value Factors Defined

Each factor within severity (expanded on the right) and value (strategic importance, population impact, provider criticality) is scored 1-5 universally[1]

2 Scores are Weighted

Weightings are determined by strategic priorities of VCH with CMO[2]

3 Calculate Final Score

Add weighted scores together to get a single severity score 1-5

Severity Scoring Guidelines[3]

Guideline	Score criteria
Non-compliance risk (20%)	1 No late/missing deliverables, no breaches.
	5 Repeated serious breaches; CAP not resolved; regulatory flag.
Performance risk (20%)	1 Consistently meets ≥95% of KPIs.
	5 Meets <50% of KPIs; serious ongoing failure.
Procurement & legal risk (15%)	1 Competitively procured, standard terms, low sensitivity.
	5 No competitive process, high regulatory exposure, active legal/audit issue.
Financial risk (25%)	1 Audited surplus; <20% of revenue from VCH.
	5 Large deficit; >80% of revenue from VCH; going-concern risk.
Reputational risk (20%)	1 No complaints/adverse media in last 24 months.
	5 Ongoing serious investigation or public controversy.

Example: Severity Scoring for VAHS and Veolia[3]

- VAHS: Mostly 1–2 across legal and financial risk → weighted score ≈ 1.75
- Veolia: Higher non-compliance and legal risk (3–4) → weighted score ≈ 3.20

These scores place VAHS top left and Veolia bottom right on Slide 2, both Tier 3

Sources: [1]Vanta, [2]Appendix A4, [3]Table generated with the aid of Claude using standardized human inputs, [3]Cesinet

Appendix | Tier Metrics Weights

Each function scores their own component, and then CMO synthesize into one agreed score



Contract advisors



Finance team



Operations team



Legal team



Subject matter experts

Severity Weights

- **Financial risk**[1] – 25% | Provider financial failure is the most direct and immediate cause of service disruption. In healthcare vendor risk, financial instability is consistently ranked as a top-tier risk because when a provider collapses, VCH inherits the problem overnight with no warning
- **Non-compliance risk**[2] – 20% | Compliance failures directly signal governance breakdown and regulatory exposure. In public healthcare, non-compliance creates legal liability, audit findings, and Ministry scrutiny
- **Performance risk**[3] — 20% | Poor performance is what patients actually feel Vendor risk frameworks weight operational/delivery performance heavily because it's the most visible failure mode
- **Reputational risk**[4] — 20% | Equal weight because VCH is a public health authority, meaning reputational damage from a provider
- **Procurement and legal risk**[5] — 15% | Lowest severity weight because legal issues are less frequent and typically slower-moving

Value Weights

- **Strategic importance**[6] – 35% | VCH's primary obligation is mission delivery, contracts that directly advance Indigenous health, equity, or core programs must receive elevated attention regardless of size
- **Population impact**[7] – 35% | Equal to strategic importance because in healthcare, scale and vulnerability of the population served is the most concrete measure of what's at stake if the contract underperforms
- **Provider criticality**[8] – 30% | Partly captured by population impact

Sources: [1]Cesinet, [2]Accountable HQ, [3]Performance Health US, [4]PwC, [5]Upguard, [6]Evaluations Hub, [7]McKinsey, [8]Credit Risk Monitor

Appendix | Why did we pick these KPIs

KPI Categories	Why Category?	Examples
Service Delivery Timely, accessible service for the intended population (appointments, hours, reach)[1]	Access and reach are core healthcare KPIs; they protect access risk, inform capacity and procurement decisions, and directly affect public and ministry accountability[1]	Median wait time from referral to first visit; % of intended population reached; clinic hours available per week in each community
Quality Clinical/service outcomes and experience, including cultural safety and satisfaction[1]	It controls clinical and safety risk, drives value or calls for redesign, makes equity and cultural safety visible[2]	% of clients with clinically significant improvement on validated scales; patient / client experience score
Financial Efficient, transparent use of funds (cost, budget adherence, unit cost)[2]	Manages financial sustainability so teams can intervene before providers hit crisis, compares similar outcomes at lower cost	% variance from approved budget, cost per unit of service, % of total funding spent on direct service
Compliance Adherence to contract terms, reporting, privacy, and legal/clinical standards[3]	Reduces regulatory and legal risk, guarantees a minimum safety floor, core domain in healthcare vendor risk checklist	% of required reports submitted on time; % of staff with up-to-date licenses

Sources: [1]Sirion, [2]Docusign, [3]LinkedIn

Appendix | RACI Matrix

RACI Matrix - Alert Escalation & Governance Pathway

Activity/Decision	Contract Advisors (CMO) Administrator	Contract Managers (CM) (Operations)	Other Ops	Finance	Legal	Indigenous & Local Service Provider SMEs	Leadership Team
1. Monitor & Trigger Alert (CLMS Dashboard)	I	R	C	C	C	C	I
2. Initial Alert Review & Assess Risk	C	R	C	I	I	C	I
3. Contact Provider & Learn Cause	C	R	C	I	I	C	I
4. Develop Corrective Action Plan	C	R	C	C	C	C	I
5. Monitor Progress to Resolution	C	R	C	C	C	C	I
6. Review CM Actions & Risk Assessment	A	C	I	I	I	I	C
7. Decide Leadership Escalation Level	A	I	I	I	C	I	C
8. Assign Owner & Timeline	A	C	I	C	C	I	R
9. Executive Escalation/Board Notification	R	C	I	I	I	I	C

Legend:

R	Responsible - Does the work
A	Accountable - Final authority
C	Consulted - Provides input
I	Informed - Kept in the loop

Notes:

- *Specialized teams (Indigenous SMEs, Finance, Legal) support CM at Step 1 and join governance at Step 2*
- *Operations triggers alert monitoring; CMs are first responders (Step 1)*
- *CMO/CA reviews CM actions and escalates if needed (Step 2)*
- *Leadership Team involved for executive-level decisions and board notifications*

Why VCH Needs Clear Roles

Contract oversight today is spread across **operations managers, contract advisors, finance, and others**, and the case notes that practices vary by person and program, which makes consistency and accountability hard

Best-practice project and governance guidance shows that when roles and decision rights are unclear, work is duplicated, key tasks are missed, and critical decisions are delayed[2]

Sources: [1]Table generated with the aid of Claude using standardized human inputs, [2]Forbes

Appendix | Future Growth Opportunities

Automation

Auto-generated contract drafts[1]

Based on service type, tier, and provider profile, the system recommends a pre-filled contract draft from the terms library

Smart renewal and renegotiation workflow

Six months before contract expiry, the system auto-generates a renewal brief summarizing KPI performance, risk scores, and benchmarks, recommending whether to renew, renegotiate, or re-procure

Predictive Analysis

Predictive contract risk scoring

Train models on past incidents to predict which contracts are likely to become at-risk in the next 6–12 months

Provider financial distress early warning[2]

Integrate dynamic external data (credit signals, public filings, sector benchmarks) with internal KPI scores

Service gap forecasting

Use demand data, population trends, and contract performance scores to predict where service gaps will emerge[2]

Why is this a long-term vision?

1. High-end tools require clean, standardized data which requires a lot of time and labor[1]
2. With CLMS, VCH is currently undergoing a lot of change which can be overwhelming for contract managers and service providers[3]

Overall, these would be high cost, high risk solutions with low immediate payoff if implemented now.

How does V.C.H. strategy set VCH up for success?

1. Standardized segmentation and KPIs to begin cleaning and transferring data into CLMS
2. CLMS-first approach to begin with what people are familiar with (Oracle)[3]
3. Hybrid approach combining human and technological analysis to show leaders the value of data-driven oversight, making it easier to justify and adopt more sophisticated tech later[4]

Source: [1]Oracle, [2]Itrex, [3]Industry Presentation, [4]Case

Appendix | Implementation through Education

SME-led Organization-Wide Training



Domain Expertise

SMEs bring deep contextual knowledge for complex, non-standardized contracts[1]



Scenario Based Training

Use real contract examples to demonstrate KPI interpretation, risk assessment, and culturally appropriate engagement



Embedded Workflow Support

SME support contract managers at all stages of the contract workflow



Weekly Office Hours

Ongoing support for troubleshooting complex cases and ensuring consistent application of framework

Goals of SME-Led Firm-Wide Training



Culturally Safe Implementation

Ensures V.C.H. framework is applied appropriately to all communities[3]



Consistent Handling of Edge Cases

Reduces variability in decision-making for contracts that cannot be fully standardized



Faster Adoption and Confidence

Contract managers gain confidence applying the framework in complex, real-world scenarios



Scalable Knowledge Transfer

Creates a repeatable model for handling new contract types and future policies[2]

Sources: 1. Training Industry, 2. The Training Associates, 3. Neovation



01

Stabilize

Dedicated tech/ process support team to answer queries and adjust dashboards

Impact: Allows users to feel confident with a named expert available



02

Review

Conduct systems and threshold reviews every 6 months

Impact: Allows users to analyze if selected KPIs are tracking risk and are at accurate benchmarks



03

Scale

Onboard high-value, high complexity teams that work closely with CMO by reusing same tiering and KPI framework

Impact: Building on system and feedback mechanisms to structured reviews and more advanced analytics

Appendix | Cost Savings Assumptions + Projections

Assumptions	Y1	Y2	Y3	Y4	Y5	Notes	Sources
Contract Growth Rate	4%						Pop Growth Rate + Increasing Elderly Pop
% high risk	30%						Estimated
Salary for Contract Manager	$100,000	$ 102,800	$ 105,246.64	$ 107,751.51	$ 110,316.00		Glassdoor
% reduction in time spent on report analysis using dashboards	10%	25%	31.5%	39.70%	50.0%	Reduce Time spent to 61%	LinkedIn
% Preventable Adverse Events	3.00%	7.00%	8.82%	11.10%	14.00%	37% of adverse events are preventable	PubMed
Assumptions	**Value**						
Average Number of Contracts Managed Per FTE	35.00						Estimated
Average Hours Spent on Metric Tracking (per day) pre-str	3.00						LinkedIn
Estimated Number of Hospitalizations Per Year	7%						Canadian Institute of Health
Canadian Pop Growth Rate	3%						Canadian Institute of Health
Cost Spent on Hospital Based Adverse Events (total Car	$685,000,000.00						PubMed
Hospital Adverse Events (Canada) - untoward/undesirab	153000						PubMed
% Reduction for risky events after proactive system	63%						PubMed
Cost Per Adverse Event	$4,477.12						Derived from Cost Spent on Adverse Events and Number of Adverse Events in Canada

Appendix | Cost Savings Projections

Admin Savings	Y1	Y2	Y3	Y4	Y5	
Number of Contract Managers	30	31.2	32.448	33.74592	35.0957568	
Contract Manager Salary	$ 3,000,000.00	$ 3,207,360.00	$ 3,415,042.97	$ 3,636,173.84	$ 3,871,623.37	
Cost Savings	$ 60,000.00	$ 801,840.00	$ 1,075,738.54	$ 1,443,561.01	$ 1,935,811.68	
Realized Risky Events						
Number of Contracts	1050.00	1092.00	1166.37	1245.79	1330.63	
Number of Risky Contracts	315.00	327.60	349.91	373.74	399.19	
Prevented Risky Contracts	198.45	206.39	220.44	235.46	251.49	
Potential Cost Savings	$177,697.06	$924,024.71	$986,950.79	$1,054,162.14	$1,125,950.58	
						$4,268,785.27
					$	5,316,951.23
Realized Harm Reduction						
Number of Residents Served Every Year	1025000.00	1055750.00	1087422.50	1120045.18	1153646.53	
Number of Residents Hospitalized	69802.50	71896.58	74053.47	76275.08	78563.33	
Number of Adverse Events (7.5%)	5235.19	5392.24	5554.01	5720.63	5892.25	
Number of Preventable Adverse Events	157.06	377.46	489.86	634.99	824.91	
Potential Cost Savings	$140,631.51	$1,689,921.95	$2,193,180.70	$2,842,929.14	$3,693,246.68	
Sensitivity (45%)	$63,284.18	$760,464.88	$986,931.32	$1,279,318.11	$1,661,961.01	
						$10,559,909.97
Total	$300,981.24	$2,486,329.58	$3,049,620.64	$3,777,041.26	$4,723,723.27	
				Cumulative	**$14,337,695.99**	

Appendix | KPI Calculations

KPI					
Hours Saved From Admin Savings					
Number Of Contract Advisors	30	31.2	32.448	33.74592	35.0957568
Average Hours Worked Yearly on Metric Tracking (pre strat)	22500.00	23400.00	24336.00	25309.44	26321.82
Hours Saved From Admin Savings	4500.00	5850.00	7665.84	10047.85	13160.91
					41224.60
Enhancing VCH Goals					
Value	**Unit**				
% of Workflow Where Checkpoints are in place for Indigenous and Local Providers	99%				
% Success of Trainings (HBS)	87%				
Success Rate for Considering Local and Indigenous Prov	86.1300%				
Contracts with Indigenous Providers	25.2				
Success Rate for Indigenous Contracts	21.70476				
Approx Rate of Members Per Contract	971.4285714				
Sensitivity Analysis	97.14285714				
Quality Care Provided to Indigenous Members	2108.4624				

	Y1	Y2	Y3	Y4	Y5	
Cost Savings	$300,981.24	$2,486,329.58	$3,049,620.64	$3,777,041.26	$4,723,723.27	$14,337,695.99
Costs	$617,493.27	$246,464.53	$192,734.32	$171,676.82	$167,784.91	$1,396,153.84
Net	-$316,512.03	$2,239,865.06	$2,856,886.33	$3,605,364.44	$4,555,938.35	
					ROI	10.27

Appendix | Cost Assumption

Assumptions	Y1	Y2	Y3	Y4	Y5
Value					
Costs					
Dashboard Maintenance	0%	30%	25%	20%	20%
Contract Advisor Salary	100000				
Training and Onboarding (of initial costs)	100%	100%	80%	70%	60%
Onboarding Initial Contracts	$ 21,000.00	$5,250.00			
New Contract Onboarding		42.00	44	45	47
CM Hours Spent Tracking Metrics	$ 40.00	240.00	240	228	216
Total # of Pages in Contract	$ 58,800.00	17640.00	3058	3180	3307
Assumptions					
Data Infrastcuture and Storage	7%				
Pages Per Contract	70				
Hours for Onboarding Initial Contracts	525	840			
Price Per Contract Onboarding	$ 200.00	$ 58,800.00			
Salary Per Day	$400.00				
Salary Per Hour	$50.00				
OCI Document Storing (per 1000 pages)	$ 1.97				

Appendix | Cost Projection

Costs	Y1	Y2	Y3	Y4	Y5
One Time Build for Dashboard	$ 342,500.00				
Integration with Existing CLMS	$ 10,000.00				
CM Dashboard Testing	$ 4,000.00				
Dashboard Maintenance	$ -	$ 102,750.00	$ 85,625.00	$ 68,500.00	$ 68,500.00
OCI SetUp	$ 1,500.00	$ -	$ -	$ -	$ -
OCI Management Fees	$ 105.77	$ 34.68	$ 6.01	$ 6.25	$ 6.50
Data Infrastcuture and Storage	$ 22,262.50	$ 22,792.35	$ 23,334.81	$ 23,890.17	$ 24,458.76
Initial Contract Onboarding	$ 21,000.00	$5,250.00			
Contract Onboarding		$8,400.00	$ 8,736.00	$ 9,085.44	$ 9,448.86
CM Staff Time on Tracking Metrics	$ 2,000.00	$12,000.00	$ 12,000.00	$ 11,400.00	$ 10,800.00
SME Training	$ 10,000.00	$ 10,000.00	$ 8,000.00	$ 7,000.00	$ 6,000.00
Training (Internal)	$ 30,000.00	$ 30,000.00	$ 24,000.00	$ 21,000.00	$ 18,000.00
Training and Onboarding (Service Providers)	$ 15,000.00	$ 8,000.00	$ 6,400.00	$ 5,600.00	$ 4,800.00
Compliance (15%)	$ 51,375.00	$ 15,412.50	$ 3,082.50	$ 3,155.86	$ 3,230.97
Legal (10%)	$ 34,250.00	$ 10,275.00	$ 6,850.00	$ 7,013.03	$ 7,179.94
IT (20%)	$ 68,500.00	$ 20,550.00	$ 13,700.00	$ 14,026.06	$ 14,359.88
Misclenaeous Costs	$ 5,000.00	$ 1,000.00	$ 1,000.00	$ 1,000.00	$ 1,000.00
Total	$ 617,493.27	$ 246,464.53	$ 192,734.32	$ 171,676.82	$ 167,784.91
					$ 1,396,153.84

Appendix | Cost Breakdown

Value	% of Y1	% of Recurring Costs
One Time Build for Dashboard	55.47%	0%
Integration with Existing CLMS	1.62%	0%
CM Dashboard Testing	0.65%	0%
Dashboard Maintenance	0.00%	42%
OCI SetUp	0.24%	0%
OCI Management Fees	0.02%	0%
Data Infrastcuture and Storage	3.61%	9%
Initial Contract Onboarding	3.40%	0%
Contract Onboarding	0.00%	3%
CM Staff Time on Tracking Metrics	0.32%	5%
SME Training	1.62%	4%
Training (Internal)	4.86%	12%
Training and Onboarding (Service Providers)	2.43%	3%
Compliance (15%)	8.32%	6%
Legal (10%)	5.55%	4%
IT (20%)	11.09%	8%
Misclenaeous Costs	0.81%	0%

Vancouver CoastalHealth

Results

We identified 2957 titles, and included 52 studies. The median ROI for public health interventions was 14.3 to 1, and median CBR was 8.3. The median ROI for all 29 local public health interventions was 4.1 to 1, and median CBR was 10.3. Even larger benefits were reported in 28 studies analysing nationwide public health interventions; the median ROI was 27.2, and median CBR was 17.5.

ROI Benchmark

Zealand.[2] While the Harvard study's initial focus was on identifying negligence, the field evolved to highlight incidents that were potentially preventable.[3,4] In 2014, the landmark Canadian Adverse Events Study reported adverse events in 7.5% of hospital admissions, with 37% of these events deemed preventable.[5]

A decade later, the Canadian Institute for Health Information (CIHI) and the Canadian Patient Safety Institute developed algo-

summary_en.pdf). It paved the way for scaled, reproducible measurements of adverse events in Canada. The authors of the CIHI report estimated that $685 million was spent Canada-wide in 2014–2015 on hospital-based adverse events; however, no detailed costing analysis was included in the report.

A linked study provides estimates of the health system impact

Canadian Institute for Health Info and PubMed

Appendix | Dashboard Build Costs

Dashboard Tier	Price Range (2026)	Best For	Key Features
Basic Dashboard	$15,000 – $35,000+	Small businesses or specific departments.	1-3 data sources, standard charts, limited interactivity.
Mid-Range Dashboard	$35,000 – $90,000+	Growing companies with diverse data needs.	Multiple data sources, interactive visuals, role-based access.
Enterprise Dashboard	$90,000 – $250,000+	Large organizations needing a strategic BI solution.	Complex data integration, AI/ML features, high scalability, full custom UI/UX.

Source: [1]Web Mobtech

Appendix | Oracle Cloud Infrastructure Document Understanding Pricing

OCI Document Understanding

Product	Unit price	Unit
OCI Document Understanding - Custom Training (First 15 training hour)	$0.00	Training Hour
OCI Document Understanding - Custom Training (Greater than 15 training hour)	$1.9659	Training Hour
OCI Document Understanding - Custom Document Properties (First 5,000 transactions)	$0.00	1,000 Transactions
OCI Document Understanding - Custom Document Properties (Greater than 5,000 transactions)	$1.9659	1,000 Transactions
OCI Document Understanding - Custom Document Extraction (First 5,000 transactions)	$0.00	1,000 Transactions
OCI Document Understanding - Custom Document Extraction (Greater than 5,000 transactions)	$39.318	1,000 Transactions
OCI Document Understanding - OCR (First 5,000 transactions)	$0.00	1,000 transactions
OCI Document Understanding - OCR (Greater than 5,000 transactions)	$1.3106	1,000 transactions

Source: [1]Oracle Cloud Infrastructure